MPP	MULTIDISCIPLINARY POLICY AND PROCEDURE		
Title	**ADMISSION AND DISCHARGE CRITERIA**		
Department	**Pediatric Intensive Care Unit**		
Applies to	**All PICU Nursing and Medical Staff, ER and Pediatric Ward Staff**		
Policy Code and Version Number	**MPP-PICU-001 (v3)**		
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1.0 PURPOSE

1.1. To give appropriate monitoring and care for children who are potentially having severe physiological instability requiring technical and artificial support.

1.2. To have a well-known criterion for all staff regarding admission and discharge of patients.

2.0 DEFINITION/ABBREVIATION

2.1. **PICU** - Pediatric Intensive Care Unit

2.2. MRP - Most Responsible Physician

2.3. Admission and discharge criteria for PICU provide guidelines and protocol of care, to be initiated when the patient management is accepted or discharge from unit.

3.0 RESPONSIBILITY

3.1. **PICU Consultant or Specialist** will be over all in-charge of the unit both in administration and medical care of the patients including emergency therapy like intubation ventilation, shock therapy and code blue resuscitation.

3.2. **PICU Physicians (Residents, Specialist)-** must assess the patient's condition and discuss the case with the PICU consultant before admitting patient from Pediatric ER, Pediatric ward or any outside referral from other hospital.

3.3. **PICU Head nurse** is responsible to make sure that this policy is being followed.

3.4. **Charge nurses** are responsible to accept cases, assist physician and supervise bedside nurses to all the procedure.

3.5. **Staff nurses** are responsible not only for taking care of newly admitted patients but also for arranging things for the discharge of patients.

3.6. **ER / Pediatric ward staff (Head nurse/Charge nurse)** are responsible in detailed handover process.

4.0 POLICY

4.1. Admission and Discharge of patients will be based from the criteria and upon the discretion of the PICU consultant or specialist and physicians. A system order in the patient's medical file shall be required.

4.1.1. **Admission:**

4.1.1.1. The Admission will be informed directly by the MRP or Specialist after duty hours.

4.1.1.2. For referred case from outside hospital, the referring Physician has to coordinate with the on call consultant or specialist for the acceptance.

4.1.1.3. All patients who need PICU admission must be aged between 31 days old to 14 years old.

4.1.1.4. Any pediatric patient who needs intensive monitoring should be admitted to PICU.

4.1.1.5. Patients are prioritized by level of nursing care and acuity, need for intensive medical therapy and type of illness.

4.1.1.6. Resuscitation equipment must be ready at all times.

4.1.1.7. Observation of standard precaution must be maintained.

4.1.1.8. Comprehensive endorsement using SBAR methodology must be done for all newly admitted patients.

4.1.1.9. New patients must be admitted on the bed management system of the Ministry of Health and discharge on time.

4.1.1.10. Categories of pediatric patients qualified for admission to PICU are as follows:

4.1.1.10.1. **Pediatric patients who needs close monitoring or observation which include but are not limited to:**

4.1.1.10.1.1. Pediatric Patients with severe, life-threatening or unstable cardiovascular disease such as:

4.1.1.10.1.1.1. Shock due hypovolemia, sepsis and cardiogenic.

4.1.1.10.1.1.2. Post-cardiopulmonary resuscitation, life-threatening dysrhythmias

4.1.1.10.1.1.3. Congenital heart disease with unstable cardio-respiratory status

4.1.1.10.2. **Patients with actual or potential life-threatening or unstable neurologic disease such as:**

4.1.1.10.2.1. Seizures which is unresponsive to therapy or requiring continuous infusion of anticonvulsants.

4.1.1.10.2.2. Acute and severely altered sensorium (GCS less than 8 or less than 10 and rapidly deteriorating)

4.1.1.10.2.3. Acute inflammation or infection of the spinal cord or brain with neurologic depression.

4.1.1.10.2.4. Progressive neuromuscular dysfunction requiring cardiovascular monitoring and/or respiratory support.

4.1.1.10.3. **Patients with unstable hematologic or oncology disease or active life-threatening bleeding which requires blood transfusion.**

4.1.1.10.4. **Patients with life-threatening or unstable endocrine or metabolic disease which includes:**

4.1.1.10.4.1. Severe diabetic ketoacidosis and severe electrolyte abnormalities such as:

4.1.1.10.4.1.1. Severe metabolic acidosis with pH less than 7.1

4.1.1.10.4.1.2. Severe hyponatremia less than 125 mmol per liter or hypernatremia greater than 160 mmol per liter.

4.1.1.10.4.1.3. Hyperkalemia greater than 6 mmol per liter requiring cardiac monitoring.

4.1.1.10.4.1.4. Symptomatic hypocalcemia

4.1.1.10.5. Patients with unstable gastrointestinal disease with moderate to severe gastrointestinal bleeding leading to hemodynamic or respiratory instability.

4.1.1.10.6. Postoperative pediatric patients requiring hemodynamic monitoring or extensive nursing care.

4.1.1.10.7. Patients with unstable multisystem disease such as:

4.1.1.10.7.1. Toxic ingestions and drug overdose with potential deterioration of respiratory and cardiovascular system.

4.1.1.10.7.2. Environmental injuries (near drowning, hypo/hyperthermia, stings, burns with impending airway compromise.

4.1.2. **Discharge:**

4.1.2.1. Categories of pediatric patients qualified for Discharge in PICU are as follows:

4.1.2.1.1. Must be in stable medical condition with vital signs within normal parameters.

4.1.2.1.2. Stable respiratory status (patient extubated requiring minimal oxygen support with stable arterial/venous blood gases) and airway patency.

4.1.2.1.3. Intravenous inotropic support, vasodilators and anti-arrhythmic drugs are no longer required.

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4.1.2.1.4. Neurologically- stable with normal GCS values and controlled seizures.

4.1.2.1.5. Stable patients who does not require frequent cardio-respiratory monitoring.

5.0 PROCEDURE (S)

5.1 Admission:

5.1.1 The PICU physician will assess if the patient requires intensive care based on the admission criteria.

5.1.2 The PICU physician will inform the consultant / specialist on-call and discuss the case if the patient requires PICU admission.

5.1.3 Patients who did not meet the admission criteria will be managed in the ward and will be re-assessed by the physician.

5.1.4 The admitting PICU admission will inform the PICU head/charge nurse about the incoming admission once it meets the criteria.

5.1.5 The PICU physician will provide written orders for admission and will secure consents from parents witnessed by the nurse on duty.

5.1.6 The pediatric Emergency Room/Pediatric ward nurse must inform the Head nurse/Charge nurse about the patient's condition and the required equipment to be prepared prior to shifting the patient to the unit.

5.1.7 The PICU nurse will prepare the bed and the machines required according to patient's case and condition.

5.1.8 The transferring nurse will give complete and proper endorsement to the PICU head/charge nurse along with the admitting PICU nurse.

5.1.9 The PICU nurse will do all the admission procedure based on the policy and physician's written order.

5.1.10 The Head/Charge nurse will update or transfer the patient's bed account in the Bed Management system - MOH.

5.1.11 The PICU nurse will send the PICU Bed Management logbook to the nursing supervisor.

5.1.12 Patient's data must be entered in the Admission logbook and in the PICU daily patient list and procedures must be completely documented as well in the patient's file by the admitting nurse.

5.2 Discharge:

5.2.1 The admitted patient will be assessed by the PICU consultant/specialist along with other PICU physicians during morning rounds and transcribes the plan for transfer to ward or discharge when the patient meets the discharge criteria.

5.2.2 The PICU physician will write the transfer to ward or discharge order in the Doctor's order sheet.

5.2.3 The PICU physician will inform the parents about the plan for discharge.

5.2.4 The PICU nurse will call the Social worker to coordinate with the family as well.

6.0 EQUIPMENTS

6.1. OASIS System

7.0 ATTACHMENT/FORMS

7.1. N/A

8.0 REFERENCES

8.1. DPP-PICU-0001(V2) – Admission and Discharge Criteria – PMSH, 11/5/1439

8.2. Saudi Central Board for Accreditation of Healthcare Institution (CBAHI) NICU standards, 3rd version, January 2016

9.0 REVISION CLAUSE

9.1. Used the new logo for AL-Baha Health Cluster.

9.2. Modify the policy and procedure lay out, changed the content of the Header and footer

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REVIEWAL SHEET
MEDICAL EXECUTIVE COMMITTEE

POLICY NAME:	ADMISSION AND DISCHARGE CRITERIA IN PICU	POLICY CODE:	MPP-PICU-001 (v3)
DATE REVIEWED:	7 / 12 / 2025		

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17	Mr. Hassan Ahmed Al-Zubeydi	Head, Physiotherapy Department		7-12-25
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20	Mr. Mohd Saud Al-Ghamdi	Head. Home Health Care Department		7/12/25
21	Ms. Lelane Bolinas	Medical Admin Secretary		7-12-25

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